Date:	Aug 20, 2025
To:	"Miao Xu" miao.xu@sdu.edu.cn
From:	"The Journal of Thrombosis and Haemostasis" jth@publishingsolutions.net
Subject:	Decision on submission to Journal of Thrombosis and Haemostasis

Manuscript Number: JTHA-D-25-01104
Machine learning-based early diagnosis of disseminated intravascular coagulation by hemostatic biomarkers

Dear Dr Xu,

Thank you for submitting your manuscript entitled "Machine learning-based early diagnosis of disseminated intravascular coagulation by hemostatic biomarkers" to the Journal of Thrombosis and Haemostasis for consideration.

I regret to inform you that the Journal of Thrombosis and Haemostasis will not be able to publish your manuscript "Machine learning-based early diagnosis of disseminated intravascular coagulation by hemostatic biomarkers." The reviewers had reservations about your paper, and as they raised some points that may be useful to you, we are enclosing their comments here.

We are only able to publish a minority of papers that are submitted and, for reasons of priority, we may have to decline otherwise acceptable material.

For alternative journals that may be more suitable for your manuscript, please refer to our Journal Finder (http://journalfinder.elsevier.com?dgcid=eman:jf-editor-reject-norev_email).

On a personal note, I hope that you will consider sending further manuscripts to us for review despite the adverse decision on this paper.

Kind regards,
Jerrold H Levy
Associate Editor

Suzanne C. Cannegieter
Editor in Chief
Journal of Thrombosis and Haemostasis

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Editor and Reviewer comments:

Your paper was reviewed by a reviewer with expertise in the subject and myself. Although the study examines mechanistic considerations of coagulation–anticoagulation imbalance in DIC, its reliance on highly specialized biomarkers that are not routinely available in a clinical setting, and the usefulness of providing additional prediction in a syndrome that is due to a specific disease with host variability limit its clinical applications.

Reviewer 1: The authors report: Machine learning-based early diagnosis of disseminated intravascular coagulation by hemostatic biomarkers.

1. In your predictive model, you use laboratory tests as biomarkers that are not routinely or clinically available, including thrombin-antithrombin complex, thrombomodulin, tissue plasminogen activator inhibitor complex, and plasmin-α2-plasmin inhibitor complex. As a result, this limits clinical applicability of your report.

2. You suggest these biomarkers are "routinely measured" which is not correct, as I am an ICU physician in a large academic medical center. Further, the cost of the assay kits having measured these before for studies is expensive, again as they are not routine tests.

3. Based on my review, you consider DIC as a primary condition for prediction, yet DIC is always

secondary to an underlying disease process, with sepsis being the most common cause in critically ill patients. DIC onset is driven by the progression of the primary illness, which can change rapidly.

4. You suggest early biomarker changes could guide intervention, but as a reminder, there is no evidence that modification or therapeutic approaches would improve patient outcomes.

5. As a reminder, association does not mean causality. For example, the TAT/ATIII ratio you consider a potential threshold for intervention. This is based on statistical association. As a reminder, multiple studies have previously examined ATIII levels, repletion, and evaluation.

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